

# Rash - Widespread On Drugs - Female (Child)

Algorithm 1120 | Child (0-17 years) | Gender at birth: Female

## Definition

- Suspected medication-related widespread rash assessment decomposed from NHS.UK's published rash emergency guidance.
- Child patient.
- Female.

## Initial assessment

1. [OPEN\_TEXT] What is the child's exact age, sex, weight, allergies and medical conditions, and what prescribed, over-the-counter, traditional/herbal, or injected medicine was started, stopped, or changed? Record exact name, dose, timing, last dose, and packaging if safely available.
2. [DURATION] When did the rash start relative to the medication?
3. [OPEN\_TEXT] Where is the rash and how fast is it spreading? Ask about facial/airway swelling, breathing/swallowing, faintness/floppiness, fever, pain, purple/non-fading spots, blisters/peeling, skin tenderness, mouth/eye/genital sores, facial swelling, vomiting, reduced urine, and safeguarding or dosing-error concerns.
4. [CAREGIVER\_OBSERVATION] What change has a safe parent or caregiver observed in behaviour, alertness, eating or drinking, sleep, school or play, and normal activity?

## Triage and redirect questions

### 1. Redirect → Anaphylaxis

Are throat or tongue swelling, breathing difficulty, faintness, confusion, or collapse the main concern?

Systemic allergic-reaction features require the anaphylaxis pathway rather than an isolated rash pathway.

### 1. Disposition level 100

Is there lip/mouth/throat/tongue swelling, breathing or swallowing difficulty, faintness, confusion, unusual floppiness, or collapse; a purple/non-fading rash with illness; or fever with painful skin, blisters, peeling, rapidly spreading redness, facial swelling, or sores involving the mouth, eyes, or genitals?

Airway, breathing, circulation, or responsiveness change may indicate anaphylaxis. Non-fading rash with illness may indicate sepsis, while painful blistering/peeling or mucosal involvement after medicine exposure may represent a severe cutaneous adverse reaction; all require emergency assessment.

### 1. Disposition level 70

With every emergency feature excluded, is there a new widespread rash after any medicine, supplement, traditional remedy, vaccine/injection, or dose change?

A rash appearing soon after starting a new medication warrants prompt review to assess whether the medication should be stopped, even without the source's own listed emergency signs.

## Advice

### ***Qatar emergency medicine-rash response***

- Call Qatar emergency services on 999 and do not drive. If anaphylaxis is suspected, use the child's prescribed adrenaline auto-injector immediately according to its instructions and emergency plan; antihistamine must not delay adrenaline or 999. Do not give another dose of the suspected medicine unless 999/qualified prescriber explicitly directs it, and do not give food, drink, or oral medicine if swallowing or responsiveness is impaired. Keep all packaging and a medicine list for handover. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: airway swelling, breathing difficulty, faintness, floppiness, or collapse; non-fading rash with illness; painful skin, blistering, peeling, mucosal sores, facial swelling, fever, or rapid progression.

### ***Urgent pediatric medication-rash review***

- Arrange prompt same-day in-person pediatric assessment through the Qatar route approved by governance and contact an authorized prescriber/pharmacist before the next dose when this can be done without delaying care. Do not independently restart, substitute, or permanently discontinue an essential medicine; document the suspected product as a reaction, not a confirmed allergy, until assessed. Do not use leftover antihistamine, steroid, antibiotic, or topical treatment without age/weight and interaction checks. Call 999 immediately for any emergency feature.
- Call Back If: rash spreads, becomes painful, blisters, peels, or involves mouth/eyes/genitals; fever, facial swelling, breathing/swallowing change, faintness, or floppiness.

## First aid

- Call Qatar emergency services on 999 and do not drive. If anaphylaxis is suspected, use the child's prescribed adrenaline auto-injector immediately according to its instructions and emergency plan; antihistamine must not delay adrenaline or 999. Do not give another dose of the suspected medicine unless 999/qualified prescriber explicitly directs it, and do not give food, drink, or oral medicine if swallowing or responsiveness is impaired. Keep all packaging and a medicine list for handover. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

## References

- NHS.UK, "Skin rash in children", <https://www.nhs.uk/conditions/skin-rash-children/> (page last reviewed 03 October 2024)

## Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from the cited NHS.UK pages under the Open Government Licence v3.0; it is not licensed STCC content.

Question.DispositionLevel=null marks a See More Appropriate Guideline redirect and is not a disposition row. Local extensions, destination mappings, safeguarding workflows, and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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